

RECOGNISING ANXIETY AND DEPRESSION

Person with **anxiety** will feel apprehensive about their future and have troubled thoughts

Anxiety can lead to depression

Bodily symptoms like tension and rigidity will occur during an intense spell of **anxiety**

Person with **depression** will feel hopeless about the future and imagining worst-case scenarios

Depression can lead to self-harm including suicide

Person with **depression** may not need a major trigger to experience physical changes like looking hopeless, drained, energy-less, having trouble sleeping and losing appetite

UNDERSTANDING PANIC DISORDER

Panic disorder is when a person has panic attacks. These moments of intense anxiety come with sensations you would have if you were in great danger, like a pounding heart, feeling faint, sweating, shaky limbs, nausea, chest pains, breathing discomfort and feeling of losing control. These symptoms can rise and peak quickly. The effects can be so severe that people experiencing a panic attack think they are dying. However, while being frightening and very real, they are not life-threatening.

SUICIDE

With campus and academic life being a stressful environment, students are at a heightened risk of self-harm including suicide. During the late teens and early 20s, secretion of hormones is increased and this amplifies positive and negative emotions. With many things happening at once, it includes awakening in terms of self-awareness, sexuality and relationships. Simultaneously, academic demands increase in volume and complexity. All of this can make the pressure to succeed academically and socially intolerable, and can lead to anxiety and depression. If the person does not seek help to deal with the root causes and symptoms, it can lead to suicide.

262 ROSE AVENUE, WILD OLIVE,
RIVERFALLS OFFICE PARK,
CENTURION, 0132

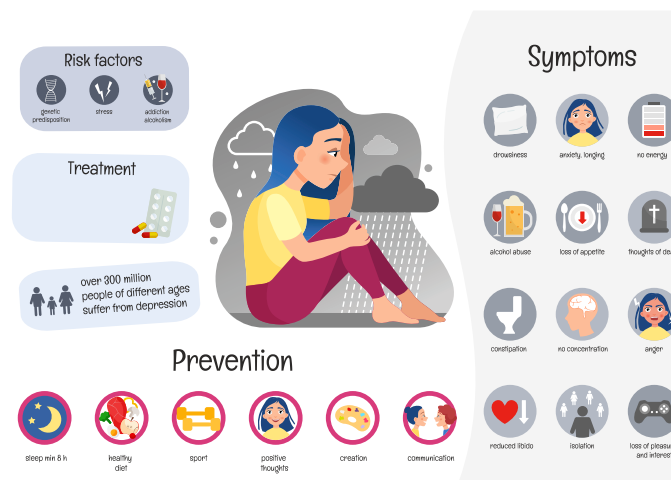
TEL +27 (0)12 030 0133

WWW.HIGHERHEALTH.AC.ZA

@HIGHERHEALTHSA

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THE FOLLOWING ARE COMMON WARNING SIGNS TO LOOKOUT FOR:



LET'S TAKE MENTAL HEALTH SERIOUSLY!

Mental health deserves the same kind of attention as our physical health. As we seek assistance when we have a fever or severe stomach cramps or another illness of physical nature, so we should get help when our emotions and feelings make us feel unwell.

Help should be available at your university or college, including peer-educators, support groups and trained professionals at the campus health centre.

Here are other services where you or a friend can get assistance:

South African Depression and Anxiety Group offers counselling and information on local support groups: 0800 21 22 23, www.sadag.org

Substance Abuse Line (run by the Department of Social Development): 0800 12 13 14, SMS 32312

Suicide Crisis Line: 0800 567 567

Akeso 24-hour Psychiatric Response Unit: 0861 435 787

Take charge of your mental health, your student life and a healthy academic career.



**PREVENTING SEXUAL HARM.
PROMOTING GENDER EQUALITY.
BUILDING MENTAL HEALTH.**



IMPROVING WELLBEING IMPROVING SUCCESS

UNDERSTANDING GENDER-BASED VIOLENCE

GBV affects individuals of all ages, genders and sexual orientation

Gender-based violence (GBV) is a violation of human rights of individuals, and a complex and wide-spread phenomenon that causes significant physical, psychological, sexual and reproductive health harm not only to people who are direct targets of GBV but communities and societies at large.

It does not target women randomly. Girls and young women are most at risk. GBV is a mirror that reflects gender stereotyping and the pattern of unequal socio-economic relations created by patriarchal cultures.

Rates of GBV in South African higher education institutions are alarming.

HOW GBV MANIFESTS WITHIN THE HIGHER EDUCATION SECTOR

These indicative behaviours occur among students and also between staff and students and between staff:

- Inappropriate and degrading comments about the appearance and sexuality of an individual
- Stalking, spying, harassment or other forms of intrusion upon an individual's personal space
- Unwanted requests for sexual engagement
- Sex-for-marks and using academic or administrative work as an excuse for inappropriate attention
- Rape and other forms of sexual assault including grievous bodily harm and murder
- Interference with an individual's studies including withholding of funding, for example the "blesser"/sugar-daddy phenomenon.

PREVENTING GBV AND RESPONDING TO VIOLENT EPISODES WITHIN THE HIGHER EDUCATION SECTOR

Meaningful programmes and services that prevent GBV on campus and assist survivors of violence combine (i) policy development and implementation and (ii) ensuring that tangible assistance – emergency response, psychosocial support, trauma health care and legal help – are accessible to all students and staff.

All institutions should also:

Adopt a zero-tolerance policy to gender discrimination and violence

Promote and teach protective and preventive strategies including gender empowerment

Provide services where survivors can access necessary assistance

Implement a safer campus programme with strengthened campus infrastructure and rapid-response mechanisms

Create peer-based teams as the first line of prevention and management of GBV.

AVOID BECOMING A TARGET

THESE ARE THE MOST PRACTICAL WAYS TO HELP YOU REDUCE THE RISK OF BEING ATTACKED:

- Be alert and aware of your surroundings – keep your eyes and ears open (avoid wearing earphones)
- Dress in clothes and shoes which can allow you free and quick movement
- Avoid walking alone through unsafe and unlit areas
- Ask people you trust to accompany you
- Organise learning groups especially for evening/night-time
- If possible invest in a pocket pepper spray/mace

DEALING WITH EMERGENCY SITUATIONS

1

BE AWARE OF YOUR NEAREST PLACE OF SAFETY/ EMERGENCY RESPONSE

2

CONSIDER LEARNING SELF-DEFENCE TO WARD OFF POTENTIAL ATTACKERS.

3

DO NOT HESITATE TO REPORT THE VIOLATION REGARDLESS OF WHO THE ATTACKER MAY BE. SPEAK TO THE CAMPUS PEER EDUCATORS OR A PERSON YOU TRUST. INFORMATION ON HOW TO GET HELP IS ALSO AVAILABLE ON WWW.GBV.ORG.ZA, CALL 0800428428, PLEASE-CALL-ME *120*7867#

CHALLENGING THE COMMON MYTHS

Myth: Individuals allow intimate partners to abuse them – they could leave if they really wanted to.

Fact: No one ever deserves abuse. Perpetrators use predatory tactics to make their target feel and remain trapped and disempowered. Partners often feel unable to leave abusive relationships due to shame and guilt, lack of safe spaces, or fear that others (eg children) will be hurt.

Myth: Conflicts and disagreements are part of any relationship. Violence, including rape of a married partner, should be tolerated for the sake of the relationship and family.

Fact: Violence in a relationship is not normal: it is a sign of unequal power relations between the partners. Every individual has the right to safety, dignity and a life free of violence. Every survivor of GBV – including partners in a marriage – should get support and protection from the justice and healthcare systems and has the right to decide whether to remain or leave the abusive relationship. Rape is defined by an action and not by the identity of the perpetrator or the survivor. Any forced sexual intercourse is rape, irrespective of whether the survivor is married to the perpetrator or not. The wellbeing of children is negatively affected when children experience or witness violence.

Myth: Violence happens to a certain type of person.

Fact: GBV is a global problem, with about a third of the women worldwide having experienced violence. While a number of factors may increase a person's risk of being a GBV target, violence can happen to anyone, regardless of age, gender, socio-economic and education circumstances.

MENTAL HEALTH AND THE HIGHER EDUCATION SECTOR



The World Health Organization (WHO) defines mental health as a state of wellbeing in which the individual realises their own abilities, can cope with the normal stresses of life, can work productively and fruitfully, and is able to make a contribution to their community.

Young people in a post-school setting face many choices, challenges and stressful situations which can affect their mental and physical wellbeing. Periodic 'blue' moods, such as grief from loss and mild sadness which are normal occurrences for nearly everyone. When these feelings become more intense and last longer, it could be a sign of a more serious condition. Depression, anxiety and substance abuse are the most common mental health problems students experience. If left untreated, they can make life seem unbearable – and contribute to self-harm including suicide.

GETTING HELP WHEN THE BLUES CAUSE INCREASED STRAIN

Studies suggest that one in four students have a diagnosable mental illness, although 40% do not seek help. Here are some questions to ask yourself and if you answer yes to any, you should talk to someone and get assistance:

- Do you experience anxious or worrisome thoughts on a daily basis?
- Are you burdened by fears others perceive as unfounded or irrational?
- Do you avoid social activities because they make you anxious?
- Do you experience sudden heart-pounding panic moments?
- Is your anxiety affecting studies, social and family life?